

LETTER OF INSTRUCTION — MEDICO-LEGAL REPORT (WC/2024/227)

Cory Lea Shepherd, Appellant (self-represented) · HOLD until the rebooked consultation

To: Dr [Krishnaiah], Consultant Psychiatrist From: Cory Lea Shepherd — Appellant, QIRC matter WC/2024/227 (self-represented) Re: Letter of instruction — medico-legal report (the "appeal report")

Dear Doctor,

I am the appellant in WC/2024/227 (Shepherd v Workers' Compensation Regulator), an appeal against the rejection of my workers' compensation claim for a psychological injury. I ask you to prepare a medico-legal report for that proceeding. This report is SEPARATE from any information provided to Metro South Health for return-to-work purposes: this report may address causation; the workplace documents must not.

I enclose: (1) a chronology of the workplace events; (2) the pleaded stressors (Amended Statement of Facts and Contentions, 7 April 2026); (3) the admitted facts extract (Form 24), including that I was rostered a 7-hour break between shifts on 17–18 March 2024 against a 10-hour minimum, and that the Regulator's own Reviewing Officer found that rostering "amounted to unreasonable management action" and was "satisfied [my] employment was a significant contributing factor"; (4) your own clinical records as you hold them.

The legal test: an injury is compensable if employment is "A SIGNIFICANT contributing factor" (s 32(1), Workers' Compensation and Rehabilitation Act 2003). Employment need not be the only or main factor; other contributors may coexist.

I ask that your report address, in your own clinical judgment:

1. Diagnosis, and the date of onset as best the clinical record establishes it (I note your record of 24 October 2024 and the escalation of medication at that time).
2. Whether my employment — in particular the pleaded matters: the rostering and fatigue events (including the 17–18 March 2024 break), the sustained workload and complaint-exposure conditions of the Switchboard role, and the non-response to the concerns and reports I raised — was A SIGNIFICANT contributing factor to the onset of the condition, and your clinical reasoning.
3. CHRONOLOGY, expressly: the diagnosis was recorded on 24 October 2024. The personal events sometimes referred to in my history (a separation and related matters) began in December 2024 and after. Please state plainly the significance, if any, those later events could have for the ONSET of a condition diagnosed before they occurred, and distinguish (if you consider it clinically appropriate) between the CAUSE of onset and factors affecting the subsequent COURSE of the condition.
4. Any clinical observations relevant to the reliability of my account — including that I continued working a reduced pattern against clinical advice through 2025, funding the shortfall from my own leave.
5. The mechanism: from a clinical perspective, how conditions of the kind described in the enclosures (sustained high demand, low control, absent support, unresolved complaints, fatigue) contribute to conditions of this kind.

Please do not address current work capacity or adjustments in this report — those are dealt with separately. I am responsible for your fee for this report; please provide an estimate. I am conscious you may be asked to produce your file in the proceeding; please write on the basis that the report and its reasoning may be tested.

Yours sincerely, Cory Lea Shepherd · 0417 400 227 · coryshepherd1@hotmail.com